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Mental Pain: A Multidimensional Operationalization and Definition

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An operationalization of mental pain is presented in three studies. The first study describes the operationalization of mental pain and the factor structure of the items produced by a content analysis of self-reports yielding a scale with nine factors: the experience of irreversibility, loss of control, narcissistic wounds, emotional flooding, freezing, estrangement, confusion, social distancing, and emptiness. Study 2 tested the relationship between mental pain and depression and anxiety in a normal population. Study 3 focused on the relationship between mental pain and coping. Mental pain is conceptualized as a perception of negative changes in the self and its functions that are accompanied by negative feelings. It is suggested that it can be meaningfully applied to the study of different mental states, life conditions, and transitions in life.

Despite the fact that philosophers, psychologists, writers, theoreticians, and laypersons alike have been fascinated by the concept of mental pain, empirical data on mental pain, a complex internal phenomenon, are scarce. In fact, in reviewing the literature on mental pain in leading psychological journals from the past 15 years, we found very few references to this concept and its operationalization.

Several attempts can be found in the literature to define mental pain. Freud (1917/1955; 1926/1959) attributed this concept specifically to feelings of mourning and longing for a beloved person following a traumatic loss. Bakan (1968) also emphasized the experience of loss as a major contributor to mental pain and viewed this to be an awareness of a disruption in the person's tendency toward maintaining a sense of wholeness and social unity. Frankl (1963) believed that mental pain and suffering are a form of emptiness due to loss of meaning in life and that it can be modulated only through adaptation of values that provide meaning to life itself.

Baumeister (1990) refered to mental pain indirectly in his theory on suicide as an escape from the self. In his view, mental pain is an aversive state of high self-awareness of inadequacy due to negative attributions to one's failures. When negative outcomes fall far below one's standards of the ideal self and aspirations, and outcomes are attributed to the self, that person experiences mental pain. It follows from Baumeister that mental pain is the heightened negative self-awareness due to the growing discrepancy between the actual and the ideal self. The basic emotion in mental pain is, thus, self-disappointment.

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Other theoreticians and investigators view mental pain from a somewhat different perspective. Herman (1992) and Janoff-Bulman (1992) imply that mental pain is instigated by trauma and loss and is, in essence, the perception of a negative change that has occurred in the self.

The most extensive contribution to the clarification of the concept of mental pain has been provided by Shneidman (1985), again within the context of a theory of suicide. Shneidman (1993, 1996) coined the term *psychache* for mental pain. He believes that mental pain is energized by frustrated or thwarted essential needs of the individual (e.g., to be loved, to have control, to protect one's self-image, to avoid shame, to feel secure, to understand) that without their satisfaction, he or she simply cannot go on living. The frustration of these needs causes a mixture of negative emotions such as guilt, shame, defeat, humiliation, disgrace, grief, hopelessness, and rage. Shneidman emphasizes, however, that the variety of negative emotions can turn into a generalized experience of unbearable mental pain—a state of emotional perturbation (Shneidman, 1980). When pain reaches a high intensity and when there is no foreseeable change in the future, the suicidal person seeks to escape the pain by committing suicide. Shneidman (1999) also provided a preliminary operationalization of mental pain by measuring the degree of need frustration and perturbation; however, this assessment procedure has not yet been validated empirically.

Recently another attempt was made to define and investigate mental pain. Bolger (1999) produced a self-narrative approach to mental pain by analyzing the scripts of the emotional pain in people who suffered traumatic experiences. Bolger's analysis yielded a definition of *mental pain* as "brokenness of the self." This sense of brokenness consists of several experiential determinants including a sense of woundedness, disconnection (from a loved one), loss of self, loss of control, and a sense of alarm. While Bolger has described and defined the experience of mental pain, she did not provide an operationalized assessment of this concept.

The conceptualization of mental pain as a basic existential condition warrants the scrutiny of this concept as a representation of a subjective state in and of itself and separate from other negative emotions and states of mind, such as depression and anxiety. In line with Shneidman (1993), we believe that pinpointing mental pain as a distinctive psychological state can be useful in the study of many aspects of behavior and subjective experiences, such as pathology, suicide, maladjustment, meaning in life, aversive life events, and therapeutic changes, as well as a wide range of personality functioning. It seems to us that Shneidman's (1999) mental pain scale (need frustration) and Baumeister's (1990) implied suggestion to measure mental pain (ideal–actual selves discrepancy; see Higgins, 1987) may be tapping only some aspects of mental pain and do not include the scope of meanings inherent in this experience.

In this article, we present several studies regarding mental pain. In Study 1 we describe the construction of a new mental pain scale using a narrative approach. In Study 2, we have scrutinized the relationship between mental pain and depression and anxiety. In Study 3, we have focused on the relationship between mental pain and coping in a normal population.

STUDY 1

Our approach to the definition and operationalization of mental pain was a narrative approach using principles of grounded theory analysis and content analysis (Glaser & Strauss, 1967; see Neimeyer & Hogan, 2000). Grounded theory analysis and content analysis can be particularly appropriate in the study of inner subjective experiences, such as mental pain. On this theoretical basis, we first analyzed narratives on mental pain; then we constructed a scale in several stages until its final version was obtained. In the present study we describe the various phases of the

scale construction and the statistical analysis of the final version.

Participants

The sample consisted of 513 Israeli Jewish adults (383 women and 129 men, ranging in age between 18 to 59, *Mdn* = 23, and in education years between 12 and 23, *Mdn* = 13) who did not receive any monetary reward for their participation in the study. Most of the participants (52%) were Israeli undergraduate students. The remaining participants were sampled from community centers in the central area of Israel.

Materials and Procedure

The participants of the present study were presented with a 44-item scale of mental pain and a list of 22 emotions to characterize their feelings when experiencing mental pain.

In a previous pilot study (Orbach & Mikulincer, 2002), we interviewed a mixed sample of 120 inpatients and normal individuals, aged 15–75. Some of the participants were randomly chosen to volunteer and others were approached because of their likelihood to experience intense mental pain (psychiatric patients, patients in general hospitals, disabled, bereaved). These volunteers were interviewed and were asked the following questions: (1) What does mental pain mean to you? (2) Have you ever experienced mental pain? (3) Can you describe this experience in detail? (4) What were the circumstances? (5) What did you feel during this experience? (6) What were your thoughts at the time? (7) Did the mental pain ever become unbearable? Participants were asked to write down their reflections on the questions in a free manner.

The content of the responses was analyzed and formalized as short statements. These statements were then structured as a scale with 220 items relating to the experiences of mental pain. Each item was rated on a 5-point scale (1 = *describes me not at all* to 5 = *describes me very well*). This scale was then administered to a new sample of 402 volunteers with similar characteristics to the first sample. The responses were subjected to item analysis, reliability, and factor analysis procedures. After the removal of low consistency and low factor-loading items (all items with factor loading below .40 were removed), an 81-item questionnaire divided into several factors was obtained (α ranging from .79 to .97). The exact formulation of the items was discussed by the four investigators until an agreement was reached (see Hill, Tompson, & Williams, 1997). This version was further subjected to an item analysis with a new sample of 258 volunteers. All items that were ambiguous or did not correlate highly with the total score of the factor, or that belonged to more than one factor, were further removed. The final scale consisted of 44 items. The instructions for this scale were as follows:

> Mental pain is an experience that is familiar to everybody. It describes what it is like when people experience anguish and suffering. We would like you to help us learn about what people go through when they experience mental pain. The following statements describe different aspects of mental pain. We would like to learn about which of these statements describe your own experience of mental pain now. Please remember that there is no right or wrong answer. What is important to us is to learn about your personal real experience. Each statement has to be rated on a scale from 1–5. If you *strongly disagree* with the statement, then circle **1**. If you *disagree* with the statement, then circle **2**. If you *agree to some extent* with the statement, then circle **3**. If you *agree* with the statement, then circle **4**. If you *strongly agree* with the statement, then circle **5**.

Participants of Study 1 also received an emotion inventory of 22 items and were asked to indicate the degree to which they experience each particular emotion when in a state of mental pain. This inventory was

subjected to a factor analysis. That factor analysis yielded 6 main factors (eigenvalue > 1) that explained 63% of the variance. Factor 1 (21% of explained variance) consisted of 4 items (loading > .40) that tap self-related negative feelings (guilt, shame, disappointment, sense of failure). Factor 2 (12%) consisted of 4 items dealing with anxiety reactions (anxiety, fear of death, dread, fear). Factor 3 (9%) consisted of 4 items dealing with depressive reactions (depression, desperation, self-pity, loneliness). Factor 4 (9%) consisted of 4 items dealing with feelings related to hurt (insulted, hurt, humiliated, betrayed). Factor 5 (7%) consisted of 3 items tapping melancholy (sadness, longing, grief) and Factor 6 (5%) consisted of 3 items tapping anger responses (anger, aggression, hate). On this basis, we computed six emotional scores by averaging items that load high on a factor. Participants were scored on the extent to which he or she felt negative self-related feelings, anxiety, depression, hurt-related feelings, melancholy, and anger in mental pain experiences.

Results

Factor Structure of the Mental Pain Scale. To examine the internal structure of the Orbach & Mikulincer Mental Pain Scale (OMMP), we conducted a factor analysis with Varimax rotation on the 44 items. This analysis yielded nine main factors (eigenvalue > 1) that explained 67.8% of the variance. These factors were the same as in the first factor analysis with the sample of 402 participants. Table 1 presents the wording of the items that loaded high in each of the nine factors (loading > .40), the percent of variance explained by each factor, and reliability coefficients (Cronbach alphas) for each factor.

As can be seen in Table 1, Factor 1 consisted of 9 items that deal with the experience of mental pain as irreversible and perpetual. On this basis, we labeled this factor "irreversibility." Factor 2 consisted of 10 items that deal with the experiential components of uncontrollability, unpredictability, helplessness, and ambiguity. We labeled this factor "loss of control." Factor 3 consisted of 5 items tapping the experiential components of hurt-related feelings, such as vulnerability, rejection, and abandonment, and we labeled it "narcissist wounds." Factor 4, "emotional flooding," consisted of 4 items tapping the experience of intense and overwhelming emotional states. Factor 5 consisted of 3 items tapping the experience of inability to react to the situation, and it was labeled "freezing." Factor 6 consisted of 3 items tapping the inability to integrate changes in self-identity, and we labeled it "self-estrangement." Factor 7, "confusion," consisted of 3 items tapping difficulties in cognitive function. "Social distancing," Factor 8, consisted of 4 items tapping approach-avoidance social orientation during the mental pain experience. Finally, Factor 9 consisted of 3 items tapping the loss of personal meaning produced by the mental pain experience, and it was labeled "emptiness." Importantly, Cronbach alpha coefficients for the nine extracted factors were acceptable, implying adequate internal consistency (see Table 1). On this basis, we computed nine factor scores by averaging items that loaded high on a factor.

To assess the reliability of the OMMP, we also conducted a test-retest examination with a 3-week interval and a new sample of students ($N = 53$; 30 females and 23 males). This time participants were asked in both administrations to relate to pain at its worst. The test-retest coefficients for the nine scales ranged between .79 and .94.

Sociodemographic Characteristics and Mental Pain Experience. A one-way multivariate analysis of variance (ANOVA) yielded a significant effect for gender on the set of the nine OMMP factors, $F(9, 503) = 2.49$, $p < .01$. Univariate ANOVAs indicated that this gender effect was significant in social distancing, $F(1, 511) = 11.74$, $p < .01$, with women reporting lower social distancing during mental pain experience ($M = 3.03$, $SD = 0.79$) than men ($M = 3.31$, $SD = 0.84$). Pearson correlations between age and the OMMP scale factors revealed significant associations only for irreversibility, $r(511) = -.16$, $p < .01$; emotional flooding, $r(511) = -.15$, $p < .01$;

TABLE 1
Loadings of OMMP Items on the Nine Extracted Factors After Varimax Rotation

Items/Factors	1	2	3	4	5	6	7	8	9
10. I have lost something that I will never find again.	.80								
44. The pain will never go away.	.72								
22. The difficult situation will never change.	.67								
26. The world has changed forever.	.67								
30. My life has stopped.	.67								
32. Something in my life was damaged forever.	.67								
34. I will never be the same person.	.63								
43. I can't change what is happening to me.	.60								
29. I will never be able to reduce my pain.	.49								
6. I am afraid of the future.		.74							
36. I have no control over the situation.		.70							
33. There is uncertainty about my life and myself.		.69							
31. I have no idea what to expect of the future.		.67							
13. I have no control over my life.		.62							
2. I am completely helpless.		.56							
28. I have no control over what is happening inside me.		.53							
9. I am completely defeated.		.48							
5. I will fall apart.		.47							
21. I cannot trust myself.		.46							
7. I am rejected by everybody.			.87						
12. I feel abandoned and lonely.			.80						
1. Nobody is interested in me.			.79						
16. Others hate me.			.69						
18. I am worthless.			.64						
14. My feelings change all the time.				.92					
35. There are strong ups and downs in my feelings.				.89					
3. I feel an emotional turmoil inside me.				.89					
8. I am flooded by many feelings.				.86					
11. I feel numb and not alive.					.76				
19. I feel paralyzed.					.75				
4. I cannot do anything at all.					.55				
17. I feel that I am not my old self anymore.						.83			
23. I feel as if I am not real.						.76			
15. I am a stranger to myself.						.73			
20. I cannot concentrate.							.75		
24. I have difficulties in thinking.							.73		
27. I feel confused.							.69		
37. I want to be left alone.								.80	
25. I need the support of other people. (R)								.74	
40. I don't feel like talking to other people.								.74	
42. I can't stay alone. (R)								.70	
41. I can't find meaning in my life.									.68
39. I have no desires.									.63
38. I have no future goals.									.40
Percent of explained variance	31.8	6.9	6.3	5.5	5.2	3.8	3.2	2.8	2.3
Cronbach Alpha coefficients	.95	.95	.93	.93	.85	.79	.80	.80	.75

Factors: 1 = Irreversibility; 2 = Loss of control; 3 = Narcissist wounds; 4 = Emotional flooding; 5 = Freezing; 6 = Self-estrangement; 7 = Confusion; 8 = Social distancing; 9 = Emptiness.

(R) = Scored in reverse.

and confusion, $r(511) = -.19$, $p < .01$. The older the participant, the lower his or her scores in these factors during mental pain experiences. Pearson correlations between education years and the OMMP factors yielded no significant association.

OMMP Factors and Emotional Reactions. A canonical correlation between the nine OMMP factors and the six assessed emotional reactions yielded four significant canonical functions. The first function, $F(54, 2543) = 18.97$, $p < .01$, was defined by the contributions of irreversibility (standardized canonical coefficient of .55) and depression (.58), which were strongly correlated between them, $r(511) = .51$, $p < .01$. The second function, $F(40, 2177) = 6.66$, $p < .01$, was defined by the contributions of loss of control (.91) and anger (.98), which were strongly associated between them, $r(511) = .53$, $p < .01$. The third function, $F(28, 1804) = 3.75$, $p < .01$, was defined by the contributions of freezing (.54) and anxiety, which were strongly associated, $r(511) = .43$, $p < .01$. Finally, the fourth function, $F(18, 1417) = 3.36$, $p < .01$, was defined by the contributions of narcissist wounds (.65) and emotional flooding (.81), on the one hand, and feelings related to hurt (1.05), on the other.

Discussion

The statistical analysis of the mental pain narratives yielded nine factors. These experiential entities share some similarities with various aspects of the definitions of mental pain discussed above. For example, aspects related to a sense of loss of self, perturbation, woundedness, and lack of meaning resonate with the factors of lack of control, emotional flooding, narcissist wounds, and emptiness in our scale, thus empowering our conception with some degree of consensual validation. There are some aspects in the present scale that are not reflected in previous definitions and these include irreversibility of pain, emotional freezing, estrangement, confusion, and need for others. In these respects, the OMMP provides a rich web of experiential qualities that define mental pain.

A careful delineation of the emerging factors of mental pain suggests that almost each factor represents one aspect of a broader category, which can be defined as the perception of a negative change in the self (e.g., "I will never be the same person") or in its function (e.g., "I have no control over my life"). Further, the moderate but significant association between these self-perceptions and the various negative emotions shows that the changes in the self and its function are related to strong negative emotions. Thus, mental pain can be conceptualized as a perception of negative changes in the self and its function that is accompanied by strong negative feelings. This definition concurs with characteristics of mental pain implied by Bolger (1999), Herman (1992) and Janoff-Bulman (1992).

STUDY 2

Mental pain as a subjective state of mind is likely to be associated and share common aspects with other aversive emotional states, such as depression and anxiety. Thus, when defining and operationalizing a concept relating to aversive inner experiences and emotions, it is necessary to examine its relationship to seemingly similar or affiliated conditions. Such an examination can reveal the degree of overlap between negative states of mind. This was the purpose of Study 2.

We chose to compare the measures of the mental pain scale with a measure of depressive and anxious cognitions. Aside from common features, the experiences of depression and anxiety may be outcomes of similar life events and conditions, such as loss and trauma that produce pain (Bolger, 1999; Freud, 1917/1955, 1926/1959; Herman, 1992). The degree of overlap or association between the construct on mental pain on the one hand, and the frequently used constructs of depression and anxiety on the other, can serve to delineate the utility of the mental pain construct.

Method

Participants. The sample consisted of 255 Israeli university students (194 women

and 61 men, ranging in age from 18 to 47, *Mdn* = 23), who volunteered to participate in the study without any monetary reward.

Instruments and Procedure. The study was conducted on an individual basis and participants were asked to complete a set of questionnaires. The order of the questionnaires was randomized across participants.

The experience of mental pain was assessed by the OMMP scale (see Study 1). Cronbach alpha coefficients for the nine OMMP factors in the present study ranged between .78 and .95, implying adequate internal consistency. On this basis, we computed nine factor scores by averaging items that belonged to a factor (see Table 1). In order to confirm the nine-factor structure, we performed a confirmatory factor analysis using the sample of Study 2. This analysis replicated the findings reported in Study 1 and validated the 9-factor structure of the scale. These nine factors explained 64.3% of the variance and the item composition of the factors was identical to that obtained in Study 1.

Anxiety and depression were assessed by the Cognition Checklist (CCL; Beck, Brown, Steer, & Eidelson, 1987; Steer, Beck, Clark, & Beck, 1994). This scale measures the frequency of automatic thoughts relevant to anxiety (12 items) and depression (14 items) separately and typifying depressed and anxious patients. It was used as an indication for degree of anxiety and depression. This scale is frequently used as a valid and reliable measure of both anxiety and depression. Higher scores reflect higher levels of automatic thoughts typical of people who are high in anxiety and depression, respectively.

Results and Discussion

Table 2 presents Pearson correlations between anxiety and depression and factors of the OMMP. As can be seen from this table, measures of anxious and depressive cognitions were significantly associated with most of the OMMP factors, with the exception of social distancing. Although these associations were significant, the OMMP factors were moderately associated with both depression (*r*s between .26 to .64, *Mdn* = .42) and anxiety (*r*s between .27 to .50, *Mdn* = .38).

TABLE 2
Pearson Correlations Between OMMP Factors and Scores of Depression and Anxiety

OMMP factors	Depression	Anxiety
Irreversibility	.54**	.42**
Loss of control	.47**	.40**
Narcissist wounds	.26**	.27**
Emotional flooding	.64**	.30**
Freezing	.42**	.51**
Self-estrangement	.40**	.36**
Confusion	.40**	.32**
Social distancing	.08	–.01
Emptiness	.43**	.37**

Note. *$p < .05$; **$p < .01$

This finding implies that the OMMP factors had a unique variance beyond their associations with anxiety and depression. A canonical correlation between the nine OMMP factors and the anxiety and depression scores yielded two significant multivariate functions, $F(18, 488) = 14.99$, $p < .01$; $F(8, 245) = 5.09$, $p < .01$, respectively. The first function was defined by the contributions of depression (standardized canonical coefficient of .87) and the OMMP factors of irreversibility (.45) and emotional flooding (.49). The second function was defined by the contributions of anxiety (1.15) and the OMMP factors of freezing (.90) and loss of control (.81). The results of this study suggest that, although mental pain, depression, and anxiety are significantly related, these emotional states do not completely overlap and are distinguishable from each other.

STUDY 3

Coping and affect regulation strategies are thoughts and behaviors intended to eliminate, maintain, or change emotional states (Baumeister, 1990; Lazarus & Folkman, 1984; Westen, 1994). Coping and affect regulation

strategies have been found to be implicated in the vulnerability for experiencing and maintaining emotional distress, such as depression (Gilboa & Gotlib, 1997; Roberts, Gilboa, & Gotlib, 1998). We assert that as coping strategies can amplify or mitigate psychological stress and negative affect, it can affect mental pain in a similar way.

Coping consists of the cognitions and behaviors that people use to assess and reduce stress and to moderate the tension that accompanies it (Billings, Cronkite, & Moos, 1983). In other words, coping is behavior designed to fill two functions: (a) problem-focused, channeling resources to solve the stress-creating problem; and (b) emotion-focused, easing the tension aroused by the threat by intrapsychic activity, such as denying or changing one's attitude toward the threatening circumstances (Lazarus & Lounier, 1978). Researchers have found that people tend to combine these two types of coping in accordance with the context and the specific problem with which they are dealing (Folkman & Lazarus, 1985), as well as with their personalities (Folkman & Lazarus, 1980).

Lazarus and Folkman (1984) proposed that the optimal coping style consists of the largest possible repertoire of coping responses. Even if intrapsychic coping aids in maintaining emotional balance, the non-use of problem-solving strategies will in the end have negative psychological outcomes. Indeed, people who are depressed tend to use intrapsychic coping at the expense of instrumental coping, whereas among relatively healthy people, the dominant style of coping is problem solving (Billings & Moos, 1981; Endler & Parkes, 1990; Folkman & Lazarus, 1980; Koff & Sangani, 1997). On this basis, we hypothesized that mental pain will be inversely related to a problem-focused coping style, whereas it will be positively related to emotion-focused coping style.

Method

Participants. One hundred Israeli university student volunteers participated in the study. They consisted of 66 females and 34 males, ranging in age from 20 to 48 (*Mdn* = 23).

Instruments and Procedure. The study was conducted on an individual basis and participants were asked to complete a set of questionnaires. The order of the questionnaires was randomized across participants. The experience of mental pain was assessed by the OMMP (see Studies 1 & 2).

Coping style was assessed with a 44-item shortened version of the Way of Coping Checklist (Folkman & Lazarus, 1980). In the current study, participants were asked to recall stressful episodes that had taken place in the past 3 months. Then, participants were presented with the 44-item scale. For each item, they were asked to indicate on a 4-point Likert scale to what extent they tend to act in the way described when confronted with the stressful events.

The scale consists of four factors: (1) Problem-Focused Coping—15 items, such as "I try to analyze the problem in order to understand it better"; (2) Emotion-Focused Coping—12 items, such as "wish that I can change how I feel," "try to look at the bright side of things," and "criticize myself"; (3) Seeking Social Support—8 items, such as "talk to someone to find out more about the situation"; and (4) Distancing—5 items, such as "I try to forget the whole thing." In the current sample, Cronbach alpha coefficients for the four coping factors were acceptable (ranging from .76 to .81), allowing us to compute four coping scores by averaging items that belonged to a factor. Higher scores reflect more reliance on problem-focused coping, emotion-focused coping, support seeking, and distancing coping.

Results and Discussion

Table 3 presents correlations between factors of the OMMS and four coping strategies: (a) Problem-focused coping was inversely associated with all the OMMP factors; (b) emotion-focused coping was positively asso-

TABLE 3
Pearson Correlations Between OMMP Factors and Coping Factors

OMMP factors	Problem-Focused Coping	Emotion-Focused Coping	Support Seeking	Distancing Coping
Irreversibility	–.42**	.50**	–.05	–.09
Loss of Control	–.41**	.35**	–.13	–.08
Narcissist Wounds	–.38**	.08	–.01	–.13
Emotional Flooding	–.23*	.35**	–.05	–.06
Freezing	–.23*	.36**	–.01	.09
Self-estrangement	–.41**	.24*	–.08	–.02
Confusion	–.38**	.27**	–.07	–.05
Social Distancing	–.21*	.02	–.55**	.09
Emptiness	–.21*	.25*	–.06	.20*

Note. *$p < .05$; **$p < .01$

ciated with most of the OMMP factors, with the exceptions of narcissist wounds and social distancing; (c) support seeking was inversely associated with the OMMP factor of social distancing; and (d) distancing coping was positively associated with the OMMP factor of emptiness.

A canonical correlation between the nine OMMP factors and the four coping scores yielded three significant multivariate functions, $F(36, 328) = 3.80$, $p < .01$; $F(24, 255) = 3.18$, $p < .01$; $F(14, 178) = 2.07$, $p < .05$, respectively. The first function was defined by the contributions of emotion-focused coping (standardized canonical coefficient of .75), on the one hand, and the OMMP factors of irreversibility (.55) and loss of control (.42), on the other. The second function was defined by the contributions of support seeking (–.83), on the one hand, and the OMMP factors of social distancing (.96), on the other. The third function was defined by the contributions of problem-focused coping (–.99) and distancing coping (.62), on the one hand, and the OMMP factors of narcissist wounds (.75), self-estrangement (.47), and emptiness (.40), on the other. The coping strategies of support seeking and social distancing were associated only with the OMMP factors of social distancing and emptiness.

The results are consistent with previous findings concerning the mitigating effects of problem-focused coping in emotional stress and affective disorders (Billings & Moos, 1981) and the positive association between emotion-focused coping and affective disorders. Although these results confirm our initial hypothesis, they point to a paradoxical situation. On the one hand, emotion-focused coping should be the needed toll to alleviate emotional pain. Emotion-focused coping in the form of emotional expression, awareness, catharsis, emotional processing, and gaining information about one's inner distress should eventuate in an emotional relief through its regulatory effects (Stanton & Franz, 1999). On the other hand, we have found that such coping is associated with increased mental pain. This paradox can be partly solved by a conceptual analysis of how emotion-focused coping is defined by the measure used in the present study. Stanton and Franz argue that such measures do not tap emotion-focused coping, but rather maladjustive behavior. Items such as "I blame myself" or "I feel bad that I cannot bypass the situation" do not represent coping; rather, they confound coping efforts with distress-laden and self-deprecatory content. It is possible that measures focusing on cognitive reappraisal, direction for action, emotional expression, and awareness would point to a more positive effect of emotional regulation. Future research should attempt to delineate the association between

adaptive strategies of affect regulation and the intensity and experience of mental pain.

GENERAL DISCUSSION

It is now widely agreed that emotional well-being goes beyond that of positive mood and lack of distress (King & Napa, 1998). Frequently proposed aspects of well-being include happiness, a sense of purpose, wisdom, creativity, a philosophy of life, achievement, and the experience of love (e.g., Becker, 1992; Rogers, 1961; Russell, 1930/1958; Ryff, 1989). Just as the construct of well-being extends beyond positive mood, the construct of mental pain emerges to be broader than the mere summation of the associated constructs of emotional distress. The proposed measure of mental pain (OMMP) emerges as a useful instrument to assess the varieties of emotional distress. These findings support Shneidman's (1993) assertion that mental pain can be defined as an experiential entity in its own right.

In the present study, the Mental Pain Scale was found to be related to depressive cognitions, anxious cognitions, and emotion-focused coping. Yet all these associations are quite moderate, showing that mental pain is a distinguishable subjective experience. The pain factors that seemed to repeatedly emerge in association with the studied variables include irreversibility, lack of control, emptiness, emotional flooding, emotional freezing, and estrangement. Taken together, our data suggest that mental pain is most appropriately defined as a wide range of subjective experiences characterized as an awareness of negative changes in the self and in its functions accompanied by negative feelings. Different factors reflect a distinct negative modification in self-image, self-definition, and self-functioning, tapping a different experiential aspect of these changes.

Our preliminary conceptualization and investigation of mental pain show that it can be distinguished from related states of mind, such as depression which is usually diagnosed, and the constellations of specific distress signs, and measured on the basis of a summation of a set of symptoms. For example, the Beck Depression Inventory (BDI), a common measure of depression severity, requires participants to endorse a list of specific feelings (e.g., sadness, pessimism, guilt), behaviors (e.g., crying, indecisiveness, loss of interest in sex), and experiences (e.g., boredom, sense of failure). In itself, the BDI does not provide a picture of the inner complexity of the subjective experience, the way OMMP is designed to do; however, in future studies, relationship between factors of mental pain and measures of depressive symptoms (such as the BDI), anxiety symptoms, as well as those of hopelessness and helplessness, need to be examined.

The OMMP scale, at face value, provides a wide, multifaceted picture of interrelated complexities that make up the experience of mental pain to provide a "feel" for one's subjective experience. The OMMP takes into account several facets not considered by the usual depression and distress measures in a relatively short scale. It may be also helpful in examining the differences among groups of individuals who score similarly on standard measures of distress, and can be thought of as a crossing junction to examine differences in personality processes, such as affect regulation, coping, defensive behavior, and ways of adjustment. Finally, and most importantly, it can serve to study the role of mental pain in suicidal behavior.

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